



University of Al-Ameed
College of Medicine

**Department of
Internal Medicine**



**General Examination
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Objectives

► Remembering

- Recall steps in a clinical approach to diagnosis./ List components of general & systemic examination./Identify six fixed general signs.

► Understanding

- Explain importance of history & general examination./Describe features of pallor, jaundice & cyanosis./Differentiate between central & peripheral cyanosis.

► Applying

- Demonstrate patient approach & hand hygiene./Measure vital signs & oxygen saturation./Perform inspection to recognize abnormalities.

► Analysing

- Distinguish between localized & generalized oedema./Analyse patient environment & correlate with health status./Compare different examination sequences.

► Evaluating

- Judge importance of general vs systemic exam./Prioritize investigations based on signs./Critically assess pathological causes of clinical signs.

► Creating

- Plan a full general examination of a new patient./Develop a personal clinical checklist.
- Design a case presentation linking signs with diagnosis.

Clinical approach

Steps to be taken to reach to diagnosis

- It involves taking full history & doing full physical examination, plus performing some investigations if needed.
- Physical examination involves General & systemic
- General examination is far more important than systemic one.

Components of general examination

- Greet the patient & relative.
- Introduce yourself.
- Explain to the patient what you are going to do.
- Clean your hand (Hand washing, sterilizing , wearing gloves ...).
- Ask about any pain (site, severity ...)

Use your eyes & ears

- ▶ 1st of all : take a good look at the patient & surroundings : patient is bed-ridden, comfortable , any drugs on the table, any monitors or machines (e.g. nebulizer) , patient position in bed , use of Oxygen, use of life support
- ▶ Take a quick look to the patient : age, Gender, facies, any obvious abnormality (e.g. limb defect , injury) , eye contact, able to talk or not (GCS) , level of consciousness , any obvious sounds (e.g. Wheeze, cough ...) , able to walk or not, able to move.. Etc.

Sequence of Examination

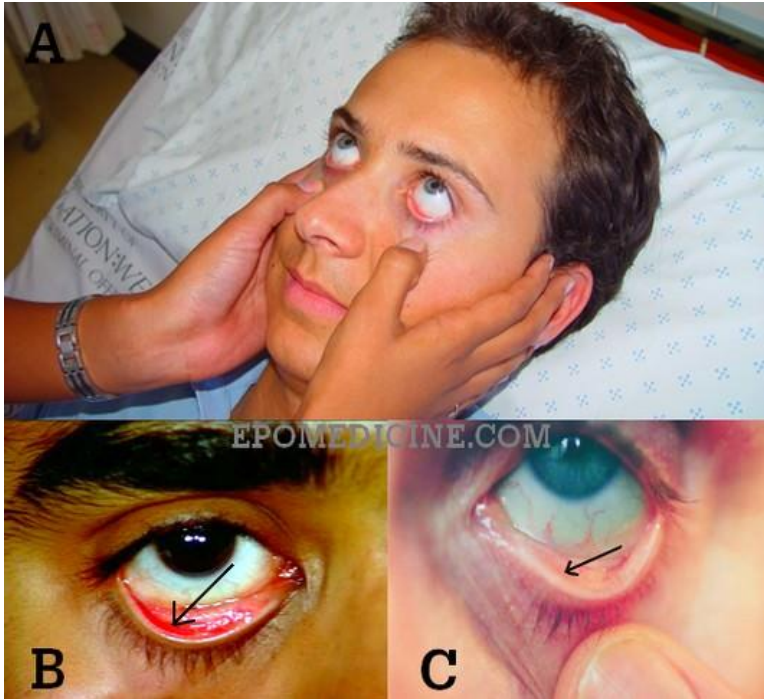
- ▶ Usually starts from hand, up to pulse, BP , Face ... then down to Neck , chest, abdomen , Lower limb.
- ▶ Other sequence : from Face going down to Limbs.
- ▶ Either talk through the examination.... Or examine silently then summarize your findings at the end.

Components of General Examination

- ▶ Vital signs : Temperature, Pulse, BP , Respiratory rate .
- ▶ SPO2
- ▶ 6 fixed general signs :
 1. Pallor
 2. Jaundice
 3. Cyanosis
 4. Edema
 5. Clubbing
 6. Lymphadenopathy

Pallor

- ▶ Definition : Pale color of the skin...
Mainly face.
- ▶ Pallor is seen in : Palm creases ,
Mucous membrane of mouth ,
Conjunctiva .
- ▶ Causes : Anemia , Fear , severe pain
....





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Jaundice

- ▶ Yellowish discoloration of skin & mucous membrane.
- ▶ Occurs when TSB exceeds threshold for age.
- ▶ Jaundice seen in : Sclera , Skin , Under the tongue.







Cyanosis

- ▶ Bluish discoloration in skin & mucous membrane resulting from increase in desaturated Hb in blood more than 4-5 gm/dl.
- ▶ 2 types : central (real cyanosis) , seen mainly in tongue .

Peripheral (Acrocyanosis) seen in fingers, mainly due to cold.



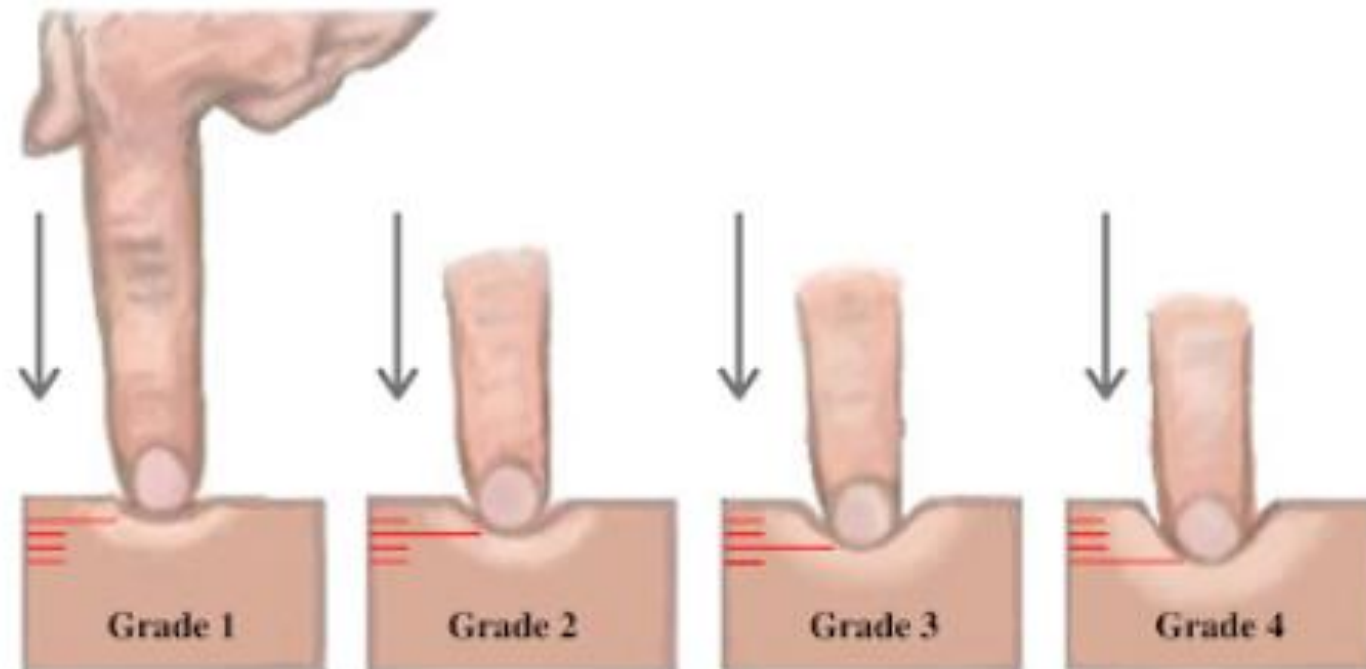


Edema

- ▶ Types : generalized & localized.
- ▶ Localized : local cause like peripheral lesion.
- ▶ Generalized : causes : Hepatic, cardiac, renal , intestinal & nutritional
- ▶ Edema Definition : Increase in amount of fluid in body tissues, mainly the Extracellular (EC) compartment.



Grade 1	0–2 mm indentation; rebounds immediately.
Grade 2	3–4 mm indentation; rebounds in < 15 seconds.
Grade 3	5–6 mm indentation; up to 30 seconds to rebound.
Grade 4	8 mm indentation; > 20 seconds to rebound.



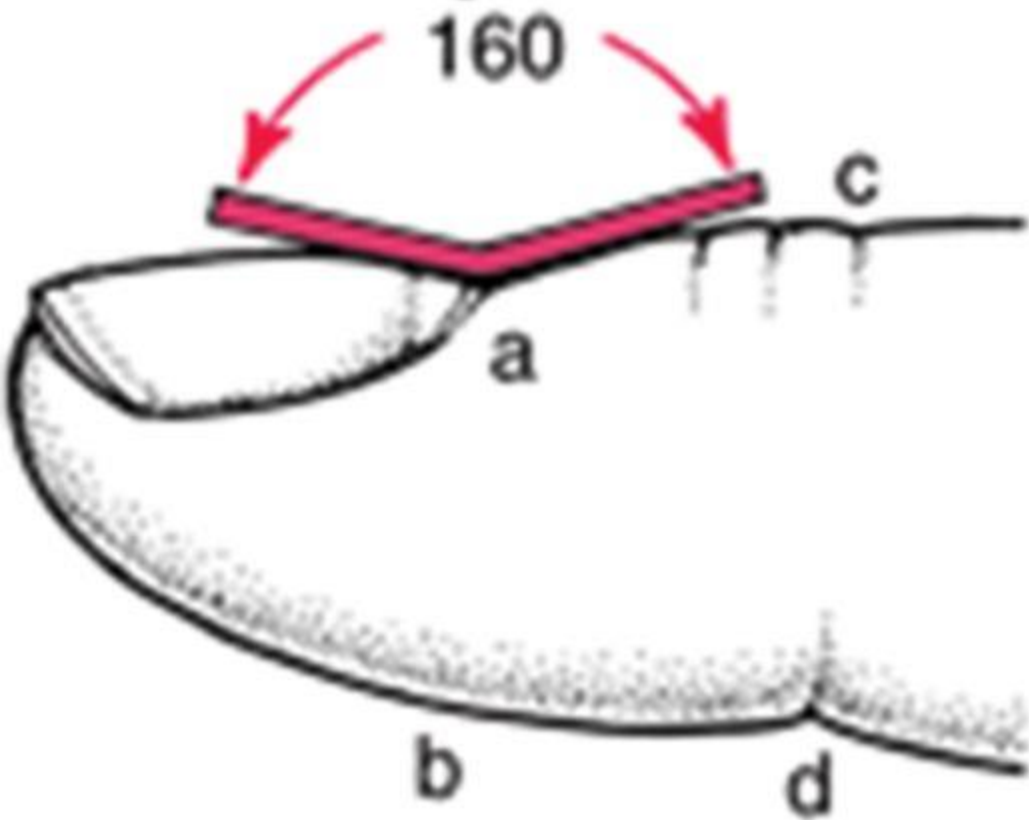


Clubbing

- ▶ Definition : deformity of the nail beds, resulting from cardiac, pulmonary , hepatic & GIT causes.

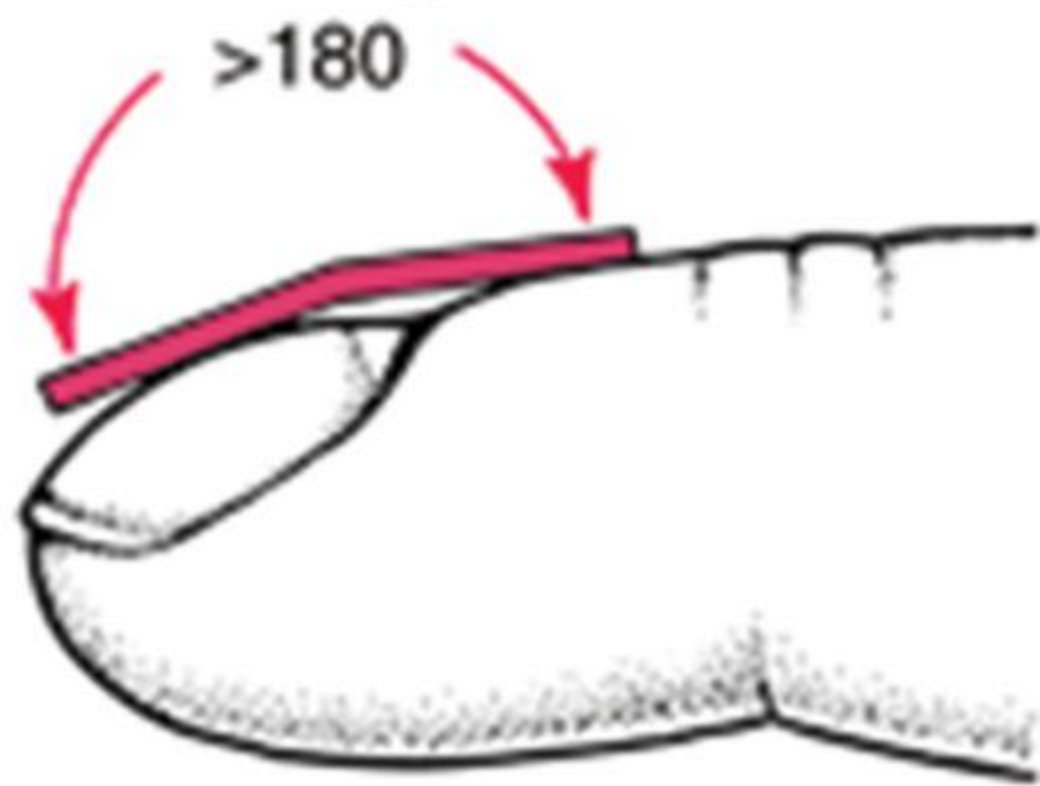
Normal finger

160

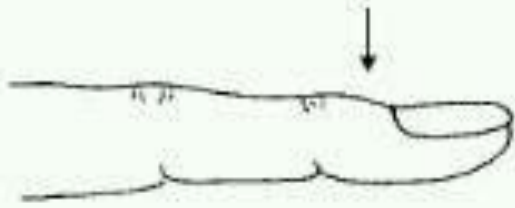


Clubbed finger

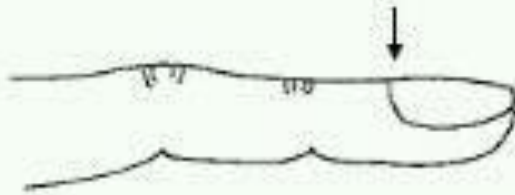
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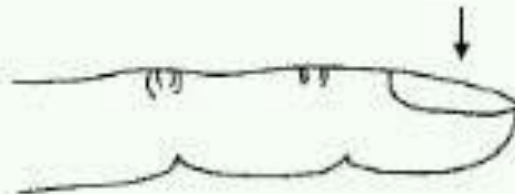
Stages of Clubbing



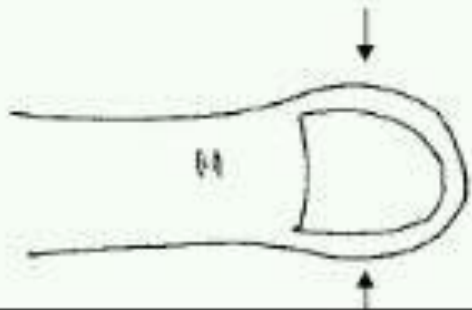
Stage 1: normal appearance and angle but increased fluctuancy of nail bed



Stage 2: loss of angle between nail and nail bed



Stage 3: increase curvature of nail



Stage 4: expansion of terminal phalanx
Drum stick appearance



Normal angle
of nail bed



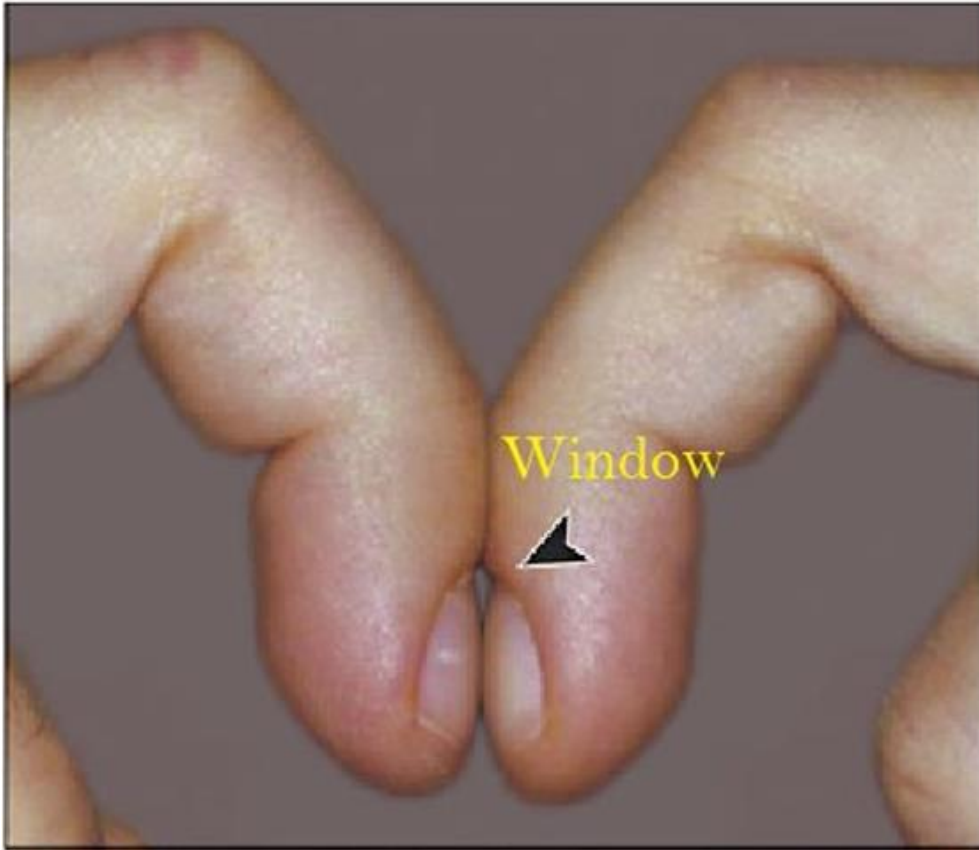
Distorted angle
of nail bed

Clubbed fingers

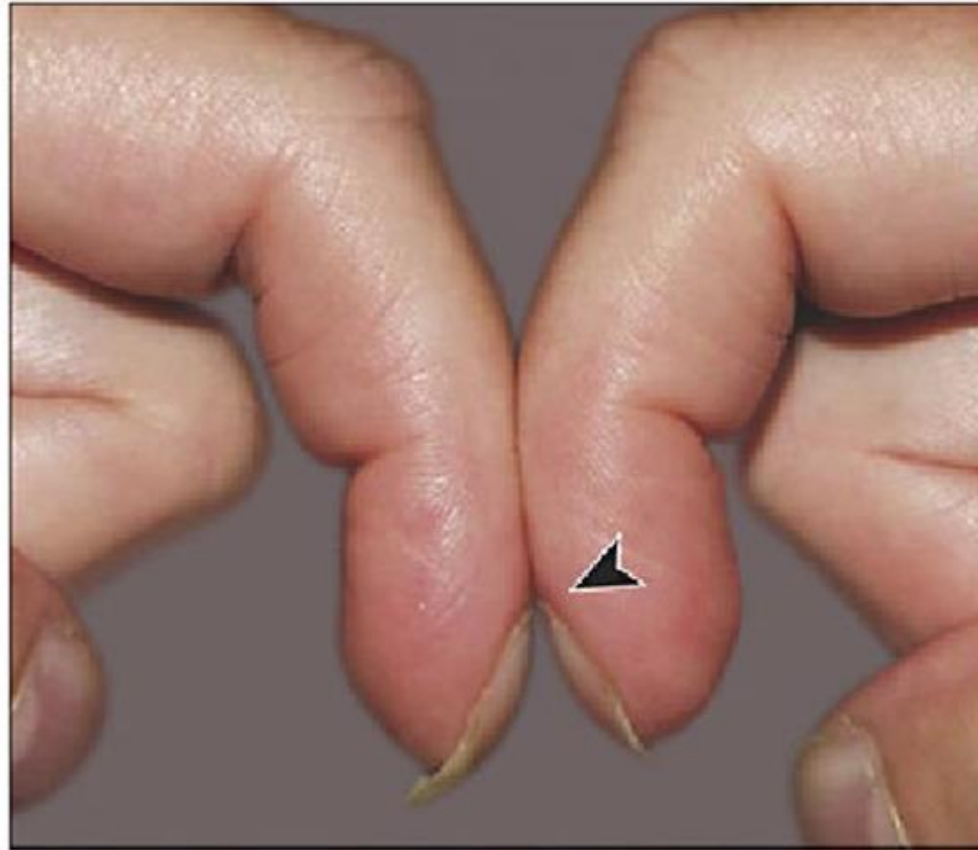


ADAM.

Normal

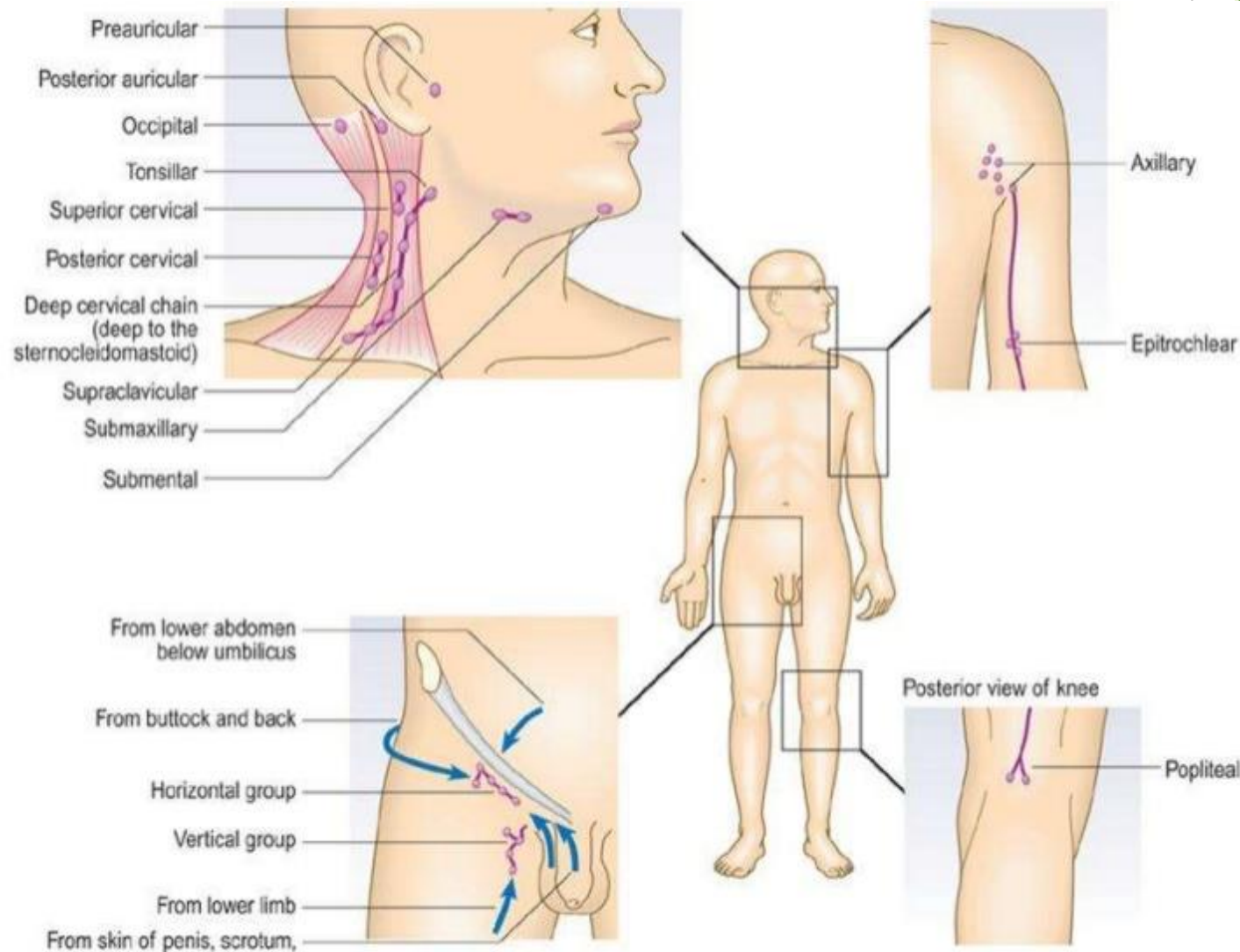


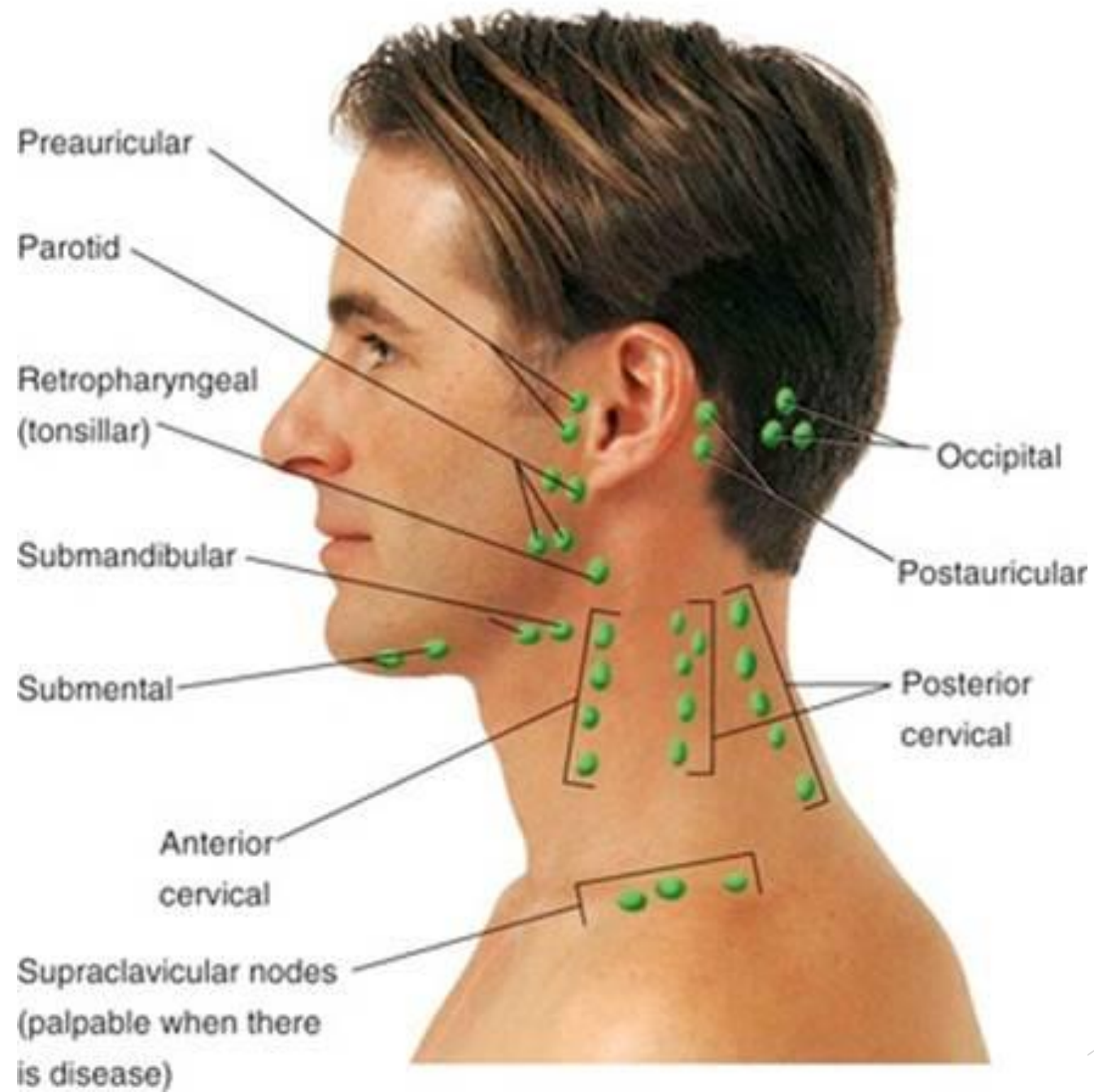
Clubbed



Lymphadenopathy

- Examining lymph node groups in the body.





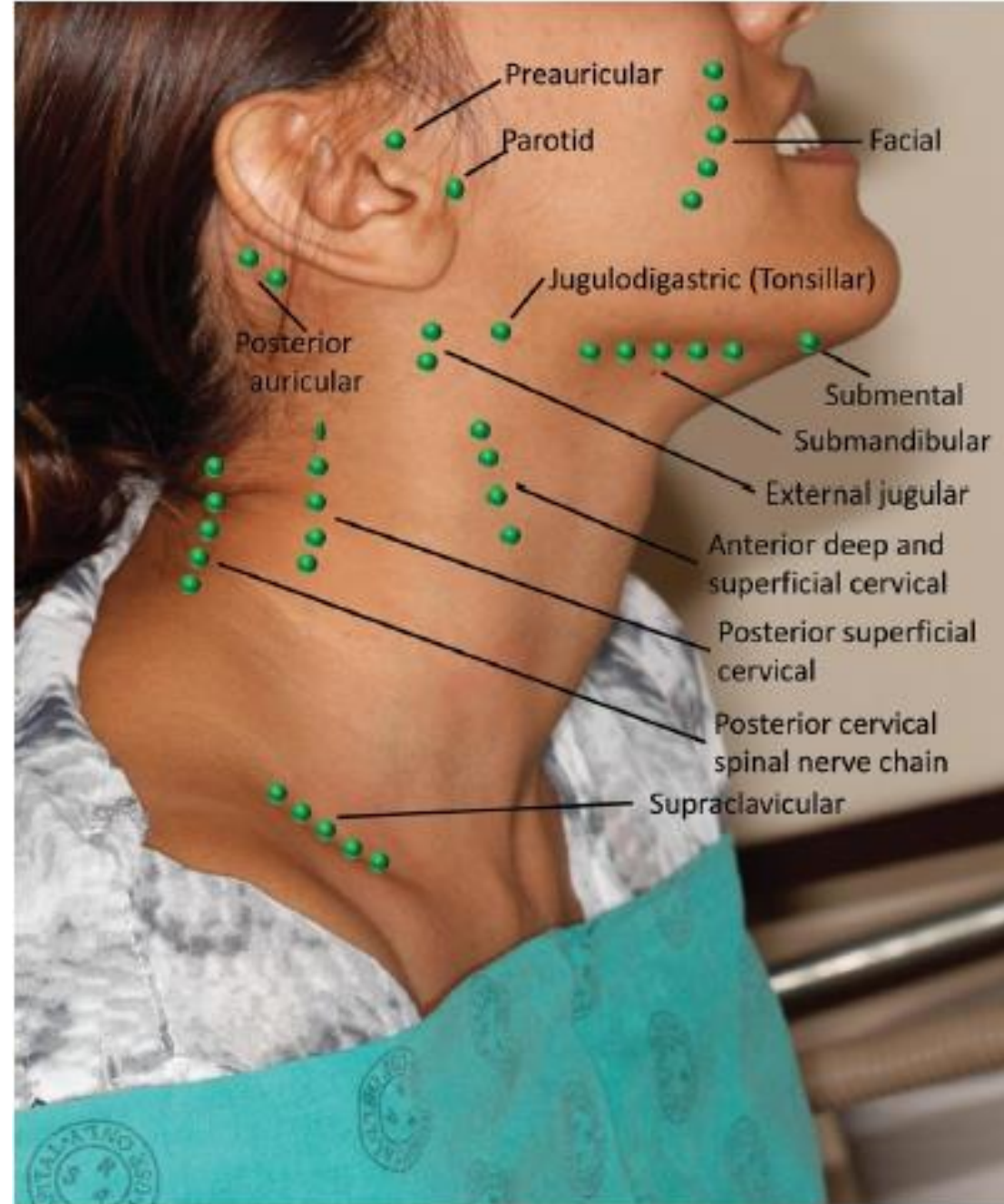


Figure 1: Clinical distribution of the more superficial and readily palpable lymph nodes of the lower facial and cervical region.

